

Healthy People 2030 (in 2026), Employment

Literature Summary & References | Source: odphp.health.gov

About This Literature Summary

This summary of the literature on Employment as a social determinant of health is a narrowly defined examination that is not intended to be exhaustive and may not address all dimensions of the issue. Please note: The terminology used in each summary is consistent with the respective references. For additional information on cross-cutting topics, please see the [High School Graduation](#) , [Housing Instability](#) , and [Poverty](#) literature summaries.

Related Objectives (3)

Here's a snapshot of the objectives related to topics covered in this literature summary. [Browse all objectives](#) .

- [Reduce the proportion of adolescents and young adults who aren't in school or working — AH-09](#)
- [Increase employment in working-age people — SDOH-02](#)
- [Increase the proportion of children living with at least 1 parent who works full time — SDOH-03](#)

Related Evidence-Based Resources (2)

Here's a snapshot of the evidence-based resources related to topics covered in this literature summary. [Browse all evidence-based resources](#) .

- [New Perspectives on Creating Jobs: Final Impacts of the Next Generation of Subsidized Employment Programs](#)
- [Strengthening TANF Outcomes By Developing Two-Generation Approaches To Build Economic Security](#)



Healthy People 2030 organizes the social determinants of health into 5 domains:

1. Economic Stability
2. Education Access and Quality
3. Health Care Access and Quality
4. Neighborhood and Built Environment

5. Social and Community Context

Literature Summary

Every day, many people are either working or looking for work.^{1, 2, 3} Multiple aspects of employment — including job security, the work environment, financial compensation, and job demands — may affect health. This summary describes how several of these aspects of employment influence health.

The Bureau of Labor Statistics (BLS) defines the labor force as individuals who are either employed (i.e., those who are working for pay or profit) or unemployed (i.e., those who are jobless but are available to work and have actively looked for work in the past 4 weeks).^{2, 4} People who are neither employed nor unemployed are not part of the labor force.^{2, 4} As of December 2019, approximately 260 million people in the United States were eligible for the labor force. Of those eligible, 63 percent participated (i.e., were employed or unemployed), and the remaining 37 percent were out of the labor force (e.g., retired).⁵

In addition, some workforce participants are underemployed, a term that indicates “involuntary part-time employment, poverty-wage employment, and insecure employment (i.e., intermittent unemployment).”⁶ Underemployment includes situations where the social status and income of a job does not match an employee’s education, abilities, and skills.^{7, 8, 9}

Harmful workplace conditions, including psychosocial stress, can increase the risk for negative health outcomes.^{1, 10, 11} In 2019, there were 2.8 million nonfatal and 5,333 fatal injuries at work.^{12, 13} Workers are prone to injuries and illness if their job includes repetitive lifting, pulling or pushing heavy loads;^{1, 14} poor-quality office equipment (e.g., keyboards and chairs);^{1, 15} long-term exposure to harmful chemicals such as lead, pesticides, aerosols, and asbestos;^{1, 16, 17} or a noisy work environment.^{1, 18, 19} In addition, highly demanding jobs and lack of control over day-to-day work activities are sources of psychosocial stress at work.¹⁰ Other sources of workplace stress include high levels of interpersonal conflict,²⁰ working evening shifts, working more than 8 hours a day, and having multiple jobs.^{1, 21} These stressors put people at risk for mortality²² and depression,²³ and they may be correlated with increased parent-child conflict and parental withdrawal.²⁴ People in highly stressful jobs may also exhibit unhealthy coping skills such as smoking or alcohol abuse.²⁵

Level of educational attainment is linked to disparities in employment because it affects the type of work people do, the working conditions they experience, and the income they earn. Workforce participants have different skill levels and educational backgrounds, which creates inequalities in wages, opportunities for advancement, job security, and other work benefits.²⁶ Individuals with less education have fewer employment choices, which may force them into positions with low levels of control, job insecurity, and low wages.⁹ Individuals with less

education are also more likely to have jobs that are physically demanding or include exposure to toxins.⁹

Other factors also influence workplace disparities. Men are more likely to work longer hours, hold higher-status jobs, and have more physically demanding jobs. However, women report more work-related physical and mental health problems.²⁷ Men tend to receive less support from coworkers and supervisors,²⁷ while women are more likely to experience sexual harassment^{28, 29} and related alcohol abuse.^{28, 30}

Racial and ethnic disparities also exist in the workforce. White people are more likely to work in white-collar clerical jobs and to assume managerial positions, while Black people are more likely to work in blue-collar service jobs.²⁶ Some racial/ethnic minority groups are also more likely to be unemployed.³¹ In October 2019, the unemployment rate for Black people was 5.4 percent, compared to 3.2 percent for White people.³¹ Additionally, Black people are more likely than White people to work in jobs that have a higher exposure to environmental risk factors and a higher risk for injury or illness.³² Workplace inequalities among racial/ethnic minority groups may cause anxiety, depression, and physical pain.³⁰

Unemployment can also have negative health consequences. Those who are unemployed report feelings of depression, anxiety, low self-esteem, demoralization,^{6, 8} worry, and physical pain.³³ Unemployed individuals tend to suffer more from stress-related illnesses such as high blood pressure,^{34, 35, 36, 37} stroke, heart attack, heart disease, and arthritis.^{8, 38, 39} In addition, experiences such as perceived job insecurity, downsizing or workplace closure, and underemployment also have implications for physical and mental health.⁸

Some strategies have been implemented to mitigate the negative health effects of unemployment and improve employee health and well-being. The Federal-State Unemployment Insurance Program provides temporary financial assistance to eligible workers who lost a job for reasons beyond their own control.⁴⁰ Although unemployment benefits vary by state laws, some research indicates that larger state unemployment benefits may alleviate poor health outcomes associated with unemployment.⁴¹ A federal agency that supports workers is the Occupational Safety and Health Administration (OSHA), which helps regulate safe work environments. OSHA outlines the rights of workers and offers opportunities for workers to request a workplace inspection or file a safety complaint.⁴² Finally, employers may offer a range of resources and benefits to improve the well-being of their employees, including health insurance, paid sick leave, and parental leave. Employer health insurance provides access to affordable medical care and financial protection from unexpected health care costs.^{43, 44} In 2019, 71 percent of civilian workers and 69 percent of private industry workers had access to health insurance, while 89 percent of state and local government employees had access.⁴⁵ Additionally, paid sick leave allows employees to seek medical care for themselves or

dependent family members without losing wages.⁴⁶ Some employers also offer parental leave after the birth of a child, which is frequently unpaid. Maternity leave has been associated with a number of positive health outcomes for both women and children.⁴⁷

Additional research is needed to better understand the beneficial effects of employment on health and to promote interventions that address disparities in employment and health. This additional evidence will facilitate public health efforts to address employment as a social determinant of health.

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